Twice-Yearly PrEP: What Changes at the Visit

Explainer video script | Written by Jonathan Troth, Ph.D. | Audience: clinicians who prescribe or could prescribe PrEP | Runtime: about 2.4 minutes (358 narration words at 150 words per minute)

Learning objectives: after watching, clinicians can (1) identify patients who may benefit from lenacapavir PrEP, (2) describe the HIV testing and dosing steps at the visit, and (3) introduce PrEP to patients in plain language.

TIME	VISUALS	NARRATION (VO)	ON-SCREEN TEXT
0:00-0:18 Hook	Title card fades to a full-year calendar grid. A pill icon drops into each day, fast, then the grid slows: a few days flicker empty, labeled with small captions (night shift, staying with family, pharmacy closed).	Pre-exposure prophylaxis, or PrEP, can prevent HIV infection. Yet for someone with inconsistent access to healthcare or who is concerned with maintaining their privacy, a daily pill represents 365 opportunities over the course of a year for life or forgetfulness to get in the way.	Twice-Yearly PrEP: What Changes at the Visit 365 doses a year
0:18-0:32 The gap	Calendar fades back. Two stat cards slide in. A meter fills to 39%; the remaining 61% stays gray.	The CDC recommends every sexually active adult be told about PrEP, yet only 39% of primary care providers in one study start conversations with their patients. The biggest barrier they cited was the lack of education.	Every sexually active adult should be told about PrEP (CDC, 2021) 39% of primary care providers start the conversation (Sell et al., 2023) Biggest barrier: lack of PrEP education
0:32-1:06 What it is	The 365 pill icons collapse into two shot icons, one in January and one in July. Then two trial cards (PURPOSE 1, PURPOSE 2) with their results, followed by an eligibility card.	In September 2025, the CDC recommended injectable lenacapavir as an effective twice-yearly alternative to the daily pill. Lenacapavir is highly effective; recent PURPOSE studies in cisgender women, and in cisgender men and gender-diverse people, show a 96 to 100% reduction in infection when compared to background rates. Lenacapavir is recommended for adults and adolescents weighing at least 35 kg who would benefit from taking PrEP. It is a suitable alternative for those who may not be able to adhere to a daily regimen.	Lenacapavir (Yeztugo): CDC recommendation, September 2025 (Patel et al., 2025) PURPOSE 1: no infections. PURPOSE 2: 96% reduction vs. background Adults and adolescents, 35 kg (77 lb) or more
1:06-1:19 HIV testing	Checklist animates one row at a time: lab test tube (check), rapid fingerstick (arrow to lab tube), oral swab (crossed out).	Lab-based antigen/antibody testing is required before the first injection and before every injection after that. If a rapid test is used, results should be confirmed via lab testing. Oral swab tests are not recommended.	Before every injection: lab-based HIV Ag/Ab test Rapid test? Confirm with a lab test Oral swab tests: not recommended
1:19-1:48 The visit	Horizontal timeline builds left to right: Day 1 (clinic icon, two shots, two tablets), Day 2 (home icon, two tablets), Month 6 (two shots, a shaded band showing plus or minus 2 weeks). Small body outline highlights the abdomen and thighs.	After HIV testing, patients begin their treatment with two subcutaneous injections and two tablets taken orally in the clinic, followed by two more tablets the next day. Going forward, patients receive two injections every six months, with a window of up to 2 weeks before or after the due date. Some soreness at the injection site is normal, yet fewer than 2 in 100 trial participants ended treatment because of injection soreness.	Day 1, in clinic: 2 injections + 2 tablets Day 2, at home: 2 tablets Every 6 months: 2 injections (window: 2 weeks either side) Injection site: abdomen or thigh Fewer than 2 in 100 stopped for injection site reactions (Patel et al., 2025)
1:48-2:23 The conversation	Return to the opening calendar, now nearly empty except for two marked days. Cut to a clinician and patient talking (photo). The	Injectable PrEP is shown to be highly effective and, by eliminating the daily need to take a pill, makes it easier	"There's now a shot you get twice a year. Would that fit better in your life?" End card: References (APA 7)

and close	final sentence appears as a speech bubble from the clinician. End card with references.	for patients to protect themselves from HIV infection. The patient with inconsistent care or concerned with privacy can access care they deserve. Start a conversation with your next patient about injectable lenacapavir to see how you can help safeguard their health. It's as simple as telling them there's now a shot you get twice a year and asking if the shot fits better in their daily lives.	

Production and accessibility notes

- Captions match the narration word for word (Section 508).
- On-screen text repeats information that is also spoken, so nothing is conveyed by visuals alone.
- Every animated element has a static end state, so the video still works as a set of stills.
- Statistics carry in-text APA citations on screen; the full reference list appears on the end card.

References

Centers for Disease Control and Prevention. (2021). Preexposure prophylaxis for the prevention of HIV infection in the United States: 2021 update. A clinical practice guideline. U.S. Department of Health and Human Services.

Gilead Sciences. (n.d.). YEZTUGO dosing and administration. Retrieved October 19, 2026, from https://www.yeztugohcp.com/dosing-administration

Patel, R. R., Hoover, K. W., Lale, A., Cabrales, J., Byrd, K. M., & Kourtis, A. P. (2025). Clinical recommendation for the use of injectable lenacapavir as HIV preexposure prophylaxis, United States, 2025. Morbidity and Mortality Weekly Report, 74(35), 541-549. https://doi.org/10.15585/mmwr.mm7435a1

Sell, J., Chen, R., Huber, C., Parascando, J., & Nunez, J. (2023). Primary care provider HIV PrEP knowledge, attitudes, and prescribing habits: A cross-sectional survey of late adopters in rural and suburban practice. Journal of Primary Care & Community Health, 14, Article 21501319221147254. https://doi.org/10.1177/21501319221147254